

Sample Language Analog Prototype

Essential Communication for Dementia Care | July 2026

Steps in the diagnosis journey

Annotated explanation of the steps on the map.

1. **Open the Conversation:** The goal of this step is to understand the concern, whose concern it is, and explore the patient's and family's current understanding of this concern. The conversation may be framed differently based on whether the patient, family, or clinician identifies the concern. If the patient is not concerned, the inquiry can be linked to prevention and general brain health, to a particular clinical situation, or as a means to allay family worry. The patient or family may have insights, experiences and emotions that need to be integrated into the conversation and approach. The patient will dictate the pace, especially if the patient's understanding of the situation does not align with what you see. The goal is not necessarily diagnosis but a common understanding that can guide next steps.
2. **Assess function:** Ask about changes in memory, mood, behavior, personality, language and function. Assess through informal conversation or a formal screen. Beginning with function grounds the conversation on the impact of cognition on daily life and can help anchor the conversation in what matters to the patient. Use a formal tool or simply ask about daily life and common activities, while listening for changes in mood, behavior, personality and language. It may be important to rule out mental health issues as part of this process.
3. **Assess cognition:** It is important to make sure the patient is willing to engage in screening or evaluation. For recognition or screening, use a brief cognitive screen or informant questionnaire. If you already have a strong suspicion of cognitive changes, you may move directly to Evaluation. For evaluation, use a validated tool to test cognitive strengths and impairment. Emphasize that this is a way to understand how different areas of the brain are functioning. After the screening or evaluation, ask the patient to reflect on their experience with the test and whether they were surprised by the results; this will inform how you frame the rest of the conversation and next steps.
4. **Assess safety:** It is important to ask both the patient and the caregiver about any immediate needs, safety concerns or risks. Probe specifically for common safety risks, including driving, cooking, falls, disorientation in familiar places, and finances. You may have to validate two different experiences or two different concerns about risks, given that dementia causes anosognosia (lack of awareness of one's impairments). You can align on shared patient and caregiver goals, such as living at home, rather than shared experiences. You may need to address any acute safety issues before the end of the visit.

5. **Targeted exam:** Perform a targeted neurological and cardiovascular exam, looking for other neurologic processes that might explain the cognitive symptoms or cardiovascular issues that can be addressed as part of protecting and enhancing vascular brain health.
6. **Labs and imaging:** Order labs and imaging to rule out other causes of cognitive changes and to look for modifiable risk factors. Imaging can also support a diagnosis of “probable dementia.” Labs can be done over time and as part of routine bloodwork if necessary. Imaging may not be required in all cases.
7. **Medication review:** Review patient medications looking for medications that can impair cognitive function, particularly psychoactive medications, sedating medications and anticholinergics. It may also make sense to use this time to simplify medication regimens if you suspect cognitive impairment is present.
8. **Assess and align understanding:** Review what is now known about function, risks to safety, cognitive strengths and challenges, and labs or imaging results. Check your understanding against the patient’s, family’s or care partner’s. Does what we’ve learned surprise them or was it expected? The goal is not diagnosis but a common understanding of any cognitive changes and their impact on the patient’s life. It is important to include patient strengths in this summary.
9. **Apply diagnosis:** Name what you think is happening. Apply as specific a diagnosis as possible with the data you have. It is okay if the patient has anosognosia or does not agree with you; you can always return to using the language around brain health. The larger goal is to maintain the connection as they go through their journey with dementia. It is important to move slowly and provide space for the emotional responses the diagnosis may elicit. People will not hear the cognitive information if they are managing strong emotions. It is also important to ask what this diagnosis means to them. This can elicit prior experience with dementia, concerns about impact on their plans for the future, and other questions. These are not necessarily problems for you to solve right now; this is part of building the groundwork for ongoing dementia care.
10. **Stage condition:** Staging the dementia as mild, moderate or advanced can help orient the patient and family and can guide decision-making.
11. **Address risks and concerns:** Prioritize patient and family concerns and any safety risks. The conversation about pharmacotherapy options may be started here or at a follow-up visit. Anticipatory guidance is dictated by the stage, rate of progression, and presence or absence of behavioral symptoms. Refer to specialists if needed.
12. **Plan follow-up:** You will be accompanying them as they adapt to and manage this new reality. If there are questions, behavioral challenges, or safety risks still to address, schedule specific follow-up to address them. If starting pharmacotherapy, this will require follow-up also. Otherwise plan for 2-3 visits in followup over the next year (integrated with other care as appropriate) to address issues as they arise. Be sure to add the diagnosis to the patient’s problem list. Communicate that we will accompany them on this journey over time.

RECOGNITION

Central questions the clinician is trying to answer at this stage

- Is there a cognitive problem present?
- What do the patient and the care partner each see about the existence of this problem?
- What do they want to do about this problem?

Goals of this stage

- Understand whether there is a concern with cognition or memory
- Explore the perspectives of the patient, family, and clinical team
- Assess for immediate or near-term risks
- Consider starting screening or evaluation if appropriate
- Manage ambiguity/set expectations: it can take a long time to get a specific diagnosis. Sometimes the answers are unclear or require time to see how things develop.
- Build trust and the relationship. Reassure the patient and family that you are here with them and there are things you can do to support them throughout the process
- Plan the next visit and next steps: address immediate risks, plan for evaluation if patient agreeable, plan to continue the conversation

Sample Language

Open the Conversation

I'd like to talk to you about your brain health. I hope to ask you some questions about any concerns you have that are related to your brain health and see how we can best support the health of your brain, as well as your body, at this time.

If Spouse/friend/family is present:

- *Thanks so much for being here today. First, I'd like to ask your [spouse/friend/family member, i.e. the patient] to share their experience with me. Then I'd like to ask you about your experience, and hear your answers to these questions. I know that we can all have different and valid viewpoints on any one situation and it will help me to hear both of your experiences.*

Patient Brings a Concern:

- *Thank you for sharing that with me. I'd like to understand more about this and make sure we are doing all we can to support your brain health.*
- *Tell me what you are noticing?*
- *Has anyone else close to you expressed concerns about your memory, mood or behavior?*
- *What do you think is going on?*

Family/Friend/Other Clinician Brings a Concern: _____ *has shared their concern that you might be having problems with memory [or having difficulty doing things that used to be easy for you]. I'd like to understand what you think about this and make sure we are doing all we can to support your brain health.*

- *What do you think about their concerns?*
- *Have you noticed any changes with your memory?*
- *What about changes in your thinking?*
- *Are there things that used to be easy for you that are now difficult?*
- *What do you think might be going on?*

Clinician Concern:

Chronic illness: *Long-standing [diabetes] can have an affect on our brain and memory. I want to make sure I am doing all I can to support your brain health as part of your [diabetes] care.*

Hospital: *It looks like you had a difficult time in the hospital. It is very common for people to experience confusion like that during a hospitalization. As your PCP, that experience tells me that we need to focus on your brain health, as well as the health of your body.*

Preventive care: *Part of my role as your PCP is to care for your whole health, including the health of your brain.*

- *I'd like to ask you some questions about your brain health.*
- *What concerns do you have about your memory?*
- *What about changes in thinking?*
- *Are there things that used to be easy for you that are now difficult?*
- *Has anyone else close to you expressed concerns about your memory, mood or behavior?*
 - *Can you share with me what those concerns have been?*

Assess Function

- *I'd like to ask you some questions about how things are going at home and in your daily life. Then we'll do a physical exam and I will order some tests.*
- *May I start with some questions?*
 - *How has your mood been lately?*
 - *Have you noticed any changes in your mood?*
- *Can you tell me a little about your daily routine?*
 - *Tell me a little about how you manage....(medications, finances, household chores/responsibilities).*
 - *Are there things you used to do but have stopped?*
 - *Or are there things you used to do easily that are now hard to do?*
 - *Can you describe what has changed?*
 - *It can be a lot to organize our daily lives. Can you tell me if you have experiences:*
 - *Missing appointments or meetings?*
 - *Missing deadlines at work or at home?*
 - *Missing or having trouble organizing your medications?*
 - *There are a lot of things we do just to get along in the modern world like driving, getting to the grocery store, shopping, cooking, using a computer. Are there things like this that you are having difficulty doing or need help with?*
 - *There are also things we do to take care of ourselves every day, like getting dressed, bathing, going to the bathroom. Where do you need help with any of these activities?*
 - *Have you gotten lost or disoriented in familiar places?*
 - *What was that like for you?*
 - *How did you find your way back?*
 - *Tell me about your social circle. How have your relationships with friends and family changed over the last year or two?*

If they mentioned memory concerns above, then ask: *Tell me how the memory issues you mentioned are affecting your daily life or your routines?*

- *Has your family noticed any changes in your daily activities?*

If Caregiver/friend/family is present:

- *Thank you for being so patient. Is there anything else right now that you'd like to add to what has been said already?*

Assess Cognition

- **For initial screening:** *I'd like to do a brief screening that can give us a little more information about your brain health. It can help us figure out what to do next. This won't give us a diagnosis, but will help us begin to understand what may be going on.*
- *Would that be okay with you?*
 - *I can do it right now and it will only take a couple of minutes. It involves drawing a clock and remembering three words.*
 - *If patient declines: "It is totally fine not to do this today."*
 - *Can I ask why you don't want to do this?"*
 - *I may ask you again if I can do it in the future.*
- **After the minicog:**
 - *How did that feel for you?*
 - *Did it surprise you at all?*

[Share results, including areas of strength, with patient and family.]

- *What do you think about these results?*
- *What surprises you?*

Assess Safety

- *I have a few more questions about common symptoms.*
 - *Have you experienced things that surprised you, like forgetting to turn off the stove?*
 - *Have you gotten lost while driving?*
 - *Have you had any falls?*
 - *Have you had any minor car accidents?*
 - *Have you had any trouble with bills, missing or forgetting to make payments?*
 - *Have you noticed any increased stress with your family, due to concerns about memory or mood changes?*
 - *[to caregiver] Do you have any concerns about the questions I just asked?*

EVALUATION

Central questions the clinician is trying to answer at this stage

- What do we know about the problem?
- How can we support the patient's strengths and identity as we engage in evaluation?
- Are there any red flags suggesting the need for specialty referral for further evaluation?

Goals of this stage

- Assess the degree of cognitive impairment and impact on function and gather the data needed to determine whether or not dementia or MCI are present
- Order and present tests and evaluation as a process of understanding brain function, both strengths and challenges, and resist focusing on a test score as having inherent meaning or prognostic ability.
- Address immediate or near-term risks and functional deficits
- Aim for a shared understanding of the situation
- Primary goal continues to be to maintain the relationship with the patient and keep them coming back, even if this prolongs the evaluation and diagnosis process
- Plan the next steps: include care partner, consider necessary referrals, offer early sharing of resources and supports for patient and care partner

Sample Language

Assess Function

- *I'd like to ask you some questions about how things are going at home and in your daily life. Then we'll do a physical exam and I will order some tests.*
- *May I start with some questions?*
 - *How has your mood been lately?*
 - *Have you noticed any changes in your mood?*
- *Can you tell me a little about your daily routine?*
 - *Tell me a little about how you manage....(medications, finances, household chores/responsibilities).*
 - *Are there things you used to do but have stopped?*
 - *Or are there things you used to do easily that are now hard to do?*
 - *Can you describe what has changed?*

- *It can be a lot to organize our daily lives. Can you tell me if you have experiences:*
 - *Missing appointments or meetings?*
 - *Missing deadlines at work or at home?*
 - *Missing or having trouble organizing your medications?*
- *There are a lot of things we do just to get along in the modern world like driving, getting to the grocery store, shopping, cooking, using a computer. Are there things like this that you are having difficulty doing or need help with?*
- *There are also things we do to take care of ourselves every day, like getting dressed, bathing, going to the bathroom. Where do you need help with any of these activities?*
- *Have you gotten lost or disoriented in familiar places?*
 - *What was that like for you?*
 - *How did you find your way back?*
- *Tell me about your social circle. How have your relationships with friends and family changed over the last year or two?*

If they mentioned memory concerns above, then ask: *Tell me how the memory issues you mentioned are affecting your daily life or your routines?*

- *Has your family noticed any changes in your daily activities?*

If Caregiver/friend/family is present:

- *Thank you for being so patient. Is there anything else right now that you'd like to add to what has been said already?*

Assess Cognition

- **For evaluation:** *I'd like to do a short evaluation which can help us understand how your brain is functioning. This doesn't measure how "smart" you are and the actual score is not important. It's more about understanding where your strengths are, where you might be experiencing some challenges, and how to adapt to the challenges.*
 - *Before we start, what are some things that have always been areas of strength for you?*
 - *What are some things that have always been challenging for you?*
- *This evaluation is designed to be a little challenging, so there may be some parts that are hard to get through. We'll get through it together.*
- *What questions do you have before we start?*

- **If patient stops midway through or refuses:** *It's okay not to finish this today. Maybe we can try it another time.*
- *Thank you for doing that with me.*

[Share results, including areas of strength, with patient and family.]

- *What do you think about these results?*
- *What surprises you?*

Assess Safety

- *I have a few more questions about common symptoms.*
 - *Have you experienced things that surprised you, like forgetting to turn off the stove?*
 - *Have you gotten lost while driving?*
 - *Have you had any falls?*
 - *Have you had any minor car accidents?*
 - *Have you had any trouble with bills, missing or forgetting to make payments?*
 - *Have you noticed any increased stress with your family, due to concerns about memory or mood changes?*
 - *[to caregiver] Do you have any concerns about the questions I just asked?*

Targeted Exam

- *I'd like to do a brief physical exam as part of this evaluation. It helps me get a more complete picture of what might be going on.*
- *Would that be okay?*
 - **If patient declines:** *It's okay not to do this today. I may ask again if we can do this at a future visit.*

Labs and Imaging

- *I'd like to check some basic labs for things that can affect the brain - these [may] include a blood count, electrolytes, vitamin B12 level, thyroid function, and liver and kidney function.*

- Unfortunately, there is no one simple test that we can use for brain health, but going through all of these steps together will help us develop a better picture of what is going on and what we can do about it.
- I also think that in your case a CT/MRI of the brain will be helpful.
- What questions do you have about these tests?
 - **If the patient declines testing:** It's fine not to do these today. I may ask you about them again in the future if I think it is important for your health.

Medication Review

- Medications, supplements, over the counter remedies - all of these can affect our brain health and our brain function. I'd like to review your medications to make sure that you are not taking anything that can affect memory and brain health.
- **Nothing concerning:** Looking at your medications, I don't see anything here that typically causes memory problems or confusion.
- **Something concerning:** One of these medications, _____, can interfere with brain health. Let's think together about whether we can reduce the dose, see how you do without it, or perhaps substitute another medication for it that has less effect on memory and thinking.

Assess and Align Understanding

- We've talked about how
 - you are having some trouble with [areas of concern]
 - **OR** [a care partner] is worried that you are having some trouble with [areas of concern].
- The [MOCA] test showed that [higher scoring sections] are areas of strength for you, places where your brain health is better preserved. It also showed that you are having [mild/moderate/significant] difficulties with...
- I also know that your [life history that may build resilience or cognitive reserve] will serve as a strength for you as we move forward.
- The labs and brain imaging studies showed some changes that are commonly seen in people with problems with memory and thinking.
- They also did not show any other cause that can explain the symptoms you are having.

NAMING & DIAGNOSIS

Central questions the clinician is trying to answer at this stage

- What do we call this problem?
- Are we naming a dementia syndrome or a disease-based diagnosis?
- Start or continue care planning and build ongoing partnership for care

Goals of this stage

- Name the current condition with as much specificity as possible and with confidence. This might include a formal diagnosis but need not. Dementia, probable Alzheimer's Disease, probably vascular dementia or mixed dementia (Alzheimer's and vascular disease simultaneously present) are the most common diagnoses in primary care.
- If there is still ambiguity in the diagnosis, name it and describe how you will manage it, either with further testing, referrals or watch and wait strategies.
- Explore and validate the response of the patient and family/caregiver.
- Align with the patient on how to talk about the current condition.
- Provide some framing about ongoing partnership and living well with dementia, adaptation to changes. It can be helpful to emphasize that there is a lot we can do to support people living with dementia.
- Respond to immediate needs and establish a clear plan for follow-up.

Sample Language

Assess and Align Understanding

- *We've talked about how*
 - *you are having some trouble with [areas of concern]*
 - **OR** *[a care partner] is worried that you are having some trouble with [areas of concern].*
- *The [MOCA] test showed that [higher scoring sections] are areas of strength for you, places where your brain health is better preserved. It also showed that you are having [mild/moderate/significant] difficulties with...*
- *I also know that your [life history that may build resilience or cognitive reserve] will serve as a strength for you as we move forward.*
- *The labs and brain imaging studies showed some changes that are commonly seen in people with problems with memory and thinking.*

- They also did not show any other cause that can explain the symptoms you are having.

Apply Diagnosis

MCI: Putting all of this information together, I think you have mild cognitive impairment.

- This means there are some changes in your memory and thinking, but they are not affecting your day-to-day activities.
- We do not know whether this will progress to dementia or not.
- In dementia, your function, or day to day activities are affected. That is not the case for you right now.
- You and I will follow this and continue to pay attention to it.
- We can also make sure we optimize your brain health now and going forward.
- I know this is a lot to take in. What questions do you have?

Dementia: Putting all of this information together, I think the cause of your symptoms is dementia.

- Dementia is a broad term for several different diseases, including Alzheimer's Disease, that cause memory loss and other changes in the brain.
- Although dementia changes over time, we don't know what it will look like for any one person.
- There is a lot we can do to support you and your family in continuing to live a fulfilling and meaningful life.
- I know this is a lot to take in. What questions do you have?
- This can be hard news to hear.
- How are you feeling right now?
- I want to make sure I am being clear in the language I am using, so can you tell me what you heard from all that I just said?
- What questions do you have?
- What other questions do you have?
- How can I best support you right now?

Stage Condition

- *I'd like to take a few minutes now and figure out what stage you are in with the changes you are noticing in your thinking and memory. This can help us to plan and can also help if you want to bring in some help to adapt to these changes.*
- *When we think about where you are in this, most people find that that they need.....*
- *There are three general stages of dementia: mild, moderate and advanced.*
 - *I think your dementia is: mild/moderate/advanced*
 - *Knowing what stage you are in helps us think about the best kinds of care and support you will need, when you need it.*

Address Risks and Concerns

- *Is there anything you are worried about right now?*
 - *What else is worrying you right now?*
- *Is there something specific you'd like more information about right now?*
- *Is there extra support that would be helpful to you right now?*
- *I don't want to lose track of something important we heard earlier -*
 - *that you are finding it increasingly difficult to...*
 - **OR** *that [care partner] was worried about...*
- *Let's talk about what we can do about that.*

Plan Follow Up

- *We have talked about a lot today. It's a lot to take in.*
- *I'd like to see you again in clinic and check back in to see what questions you have and how this is impacting you.*
- *At our next visit we may do a more complete assessment and we will start building a plan of care for you and your family.*
- *I'd also like to share some information with you today and then have you come back to discuss possible treatments.*
- *In the meantime, I want to encourage you to do the things we know are important for brain health - which include regular exercise, minimizing or stopping alcohol use, and staying socially engaged.*

- *We will continue to talk about treatments and supports at our future visits together. Our team is here to help you and your family live well, meaningfully and rewardingly with dementia.*